

# Access guide — pancreatic-mets-kras-g12d-basal-arid1a -k9r3

*How to access each therapy: trial contacts + manufacturer medical-info*

Generated 2026-07-10

# Access guide — pancreatic-mets-kras-g12d-basal-arid1a-k9r3

How a patient or treating team could practically access each intervention in this case's dossier. Trial recruitment contacts and manufacturer medical-information lines are captured for direct outreach. The number in the first column of the summary table is the entry's identifier in the per-intervention sections below — use it to find the deep section quickly. Information ages — each row carries a [Verified](#) date; re-screen before relying on a specific contact or trial slot.

## Summary

| # | Intervention                                                                                              | Modality       | Access status                            | Regulatory                                                                                                                    | Recommended first action                                                                                                                                                                         |
|---|-----------------------------------------------------------------------------------------------------------|----------------|------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 1 | pooled mutant-KRAS peptide vaccine + nivolumab + ipilimumab                                               | vaccine        | Off-label use + Clinical trial           | investigational vaccine (no approval); the nivolumab and ipilimumab backbone components are FDA-approved in other tumor types | Do not pursue off-label nivolumab + ipilimumab as a standalone strategy here; the MSS / TMB-8.6 profile predicts low benefit and payers will likely deny it                                      |
| 2 | ELI-002 (amphiphile mutant-KRAS peptide vaccine, G12D/G12R + Amph-CpG-7909)                               | vaccine        | Clinical trial + Expanded access program | investigational; no approval anywhere                                                                                         | Treat ELI-002 as informational for now; neither the trial nor the EAP fits unresectable metastatic disease                                                                                       |
| 3 | INCB161734, oral KRAS G12D-selective inhibitor                                                            | small_molecule | Clinical trial                           | investigational; no approval anywhere                                                                                         | Call the Incyte US call center (1.855.463.3463) or email <a href="mailto:medinfo@incyte.com">medinfo@incyte.com</a> to find the nearest DAWN-303 (NCT07522073) site with an open first-line slot |
| 4 | anti-KRAS G12D HLA-restricted TCR-engineered T cells (NT-112, AZD0240, and the published C*08:02 product) | CAR-T          | Clinical trial + Not yet accessible      | investigational; no approval anywhere                                                                                         | Do not pursue NCT06218914; the patient's HLA type (C05:01/C06:02, A01:01/A02:01) does not match either arm                                                                                       |
| 5 | ceralasertib (AZD6738), oral ATR inhibitor                                                                | small_molecule | Clinical trial                           | investigational; no approval anywhere                                                                                         | Have the treating team order ARID1A (BAF250a) IHC on tumor tissue; the frameshift call alone does not confirm eligibility                                                                        |

|    |                                                           |                |                                          |                                                                                                                                                                                                                             |                                                                                                                                                                        |
|----|-----------------------------------------------------------|----------------|------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| 6  | daraxonrasib (RMC-6236), pan-RAS(ON) inhibitor            | small_molecule | Clinical trial + Expanded access program | investigational; FDA breakthrough therapy designation in PDAC; NDA expected on the strength of RASolute 302 but no approval yet (as of June 2026)                                                                           | Call the Revolution Medicines study line (1-844-2-REVMED) or email medinfo@revmed.com to find the nearest RASolute 303 (NCT07491445) site with an open first-line slot |
| 7  | tuvusertib (M1774), oral ATR inhibitor                    | small_molecule | Clinical trial                           | investigational; no approval anywhere                                                                                                                                                                                       | Treat tuvusertib as secondary to ceralasertib on the ATR axis; its ARID1A cohort is endometrial-centered and the trial isn't recruiting                                |
| 8  | zoldonrasib (RMC-9805), KRAS G12D(ON)-selective inhibitor | small_molecule | Clinical trial                           | investigational; no approval anywhere                                                                                                                                                                                       | Call 1-844-2-REVMED or email medinfo@revmed.com to locate the nearest RASolute 305 (NCT07621718) site with an open first-line slot                                     |
| 9  | MRTX1133, KRAS G12D-selective noncovalent inhibitor       | small_molecule | Unavailable                              | investigational; program discontinued                                                                                                                                                                                       | Do not pursue MRTX1133 directly; the program is closed                                                                                                                 |
| 10 | camonsertib (RP-3500), oral ATR inhibitor                 | small_molecule | Unavailable                              | investigational; no approval; ATTACC combination trial terminated (no phase 2 expansion). Roche ended its license in 2024; Repare regained rights                                                                           | Do not pursue camonsertib via ATTACC; it is terminated with no successor combination trial                                                                             |
| 11 | tazemetostat (EPZ-6438), EZH2 inhibitor                   | small_molecule | Unavailable                              | previously FDA-approved (epithelioid sarcoma; relapsed/refractory follicular lymphoma); voluntarily withdrawn from all markets by Ipsen on 9 March 2026 over safety signals; all trials and expanded-access programs closed | Do not pursue tazemetostat; it has been withdrawn from all markets and all programs are closed                                                                         |

## Off-label use (1)

### 1. pooled mutant-KRAS peptide vaccine + nivolumab + ipilimumab

*Aliases:* mKRAS peptide vaccine, pooled mutant KRAS vaccine

**Modality:** vaccine · **Regulatory:** investigational vaccine (no approval); the nivolumab and ipilimumab backbone components are FDA-approved in other tumor types · **Guidelines:** the vaccine is not in any guideline; nivolumab/ipilimumab are not NCCN-recommended for MSS PDAC and checkpoint monotherapy is low-yield here given MSS, TMB 8.6 · **Geographic scope:** US (single-center academic study, completed) · **Verified:** 2026-06-06

The vaccine itself is a finished single-center academic study with no open slot and no manufacturer to supply it, so the combination as studied is not obtainable. The checkpoint backbone (nivolumab plus ipilimumab) is FDA-approved in other cancers and could in theory be prescribed off-label, but for this MSS, TMB-8.6 tumor that would be low-yield, off-guideline, and very likely denied by payers. The useful read from this row is mechanistic: the immunotherapy route into this tumor runs through KRAS-inhibitor-driven antigen restoration plus checkpoint, not checkpoint or vaccine alone.

### Next steps

1. Do not pursue off-label nivolumab + ipilimumab as a standalone strategy here; the MSS / TMB-8.6 profile predicts low benefit and payers will likely deny it
2. Confirm MMR protein IHC (MLH1/PMS2/MSH2/MSH6) on tumor tissue before any checkpoint-based claim, per the case's validation gate
3. Route the checkpoint rationale instead toward KRAS-inhibitor-plus-anti-PD-1 cohorts (e.g. the retifanlimab cohort in INCB161734 NCT06179160)
4. If a research team wants the academic protocol detail, contact [nzaidi1@jhmi.edu](mailto:nzaidi1@jhmi.edu)

### Manufacturer / sponsor contact

- **Company:** Johns Hopkins / Sidney Kimmel Comprehensive Cancer Center (academic sponsor of the vaccine); Bristol Myers Squibb markets nivolumab and ipilimumab
- **Med info phone:** 1-800-321-1335
- **Med info email:** [medical.communications@bms.com](mailto:medical.communications@bms.com)
- **Notes:** The vaccine is an academic construct with no manufacturer to dispense it. The BMS line covers the nivolumab/ipilimumab components only. Academic correspondence: [nzaidi1@jhmi.edu](mailto:nzaidi1@jhmi.edu) (N. Zaidi, Johns Hopkins).

**Payer / coverage:** The investigational vaccine has no payer pathway. Off-label nivolumab + ipilimumab for MSS PDAC would face prior-authorization denial in most plans, since there is no on-label indication, no NCCN listing, and the MSS / TMB-8.6 profile predicts low benefit from checkpoint blockade. Not a route to pursue on its own.

**Notes:** Two paths in the array: `clinical_trial_only` reflects the (now completed) academic vaccine study; `off_label_use` reflects the approved nivolumab/ipilimumab components. Neither is a recommended action for this patient given MSS biology.

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## Clinical trial (7)

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### 2. ELI-002 (amphiphile mutant-KRAS peptide vaccine, G12D/G12R + Amph-CpG-7909)

*Aliases:* ELI-002, ELI-002 7P, ELI-002 2P, Amph-Peptides, Amph-CpG-7909

**Modality:** vaccine · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 AMPLIFY-201 showed 84% mKRAS T-cell responses in the MRD/adjuvant setting · **Geographic scope:** US (expanded access protocol is US-only) · **Verified:** 2026-06-06

The KRAS vaccine axis is HLA-agnostic, so this patient's alleles don't rule it out, but the practical problem is setting, not biology: both the ELI-002 trial and its expanded-access protocol are written for the post-resection MRD/adjuvant population, and he has newly diagnosed unresectable metastatic disease. There is no enrollable ELI-002 path for metastatic disease right now. The most a treating team could do is email Elicio to ask whether any metastatic-eligible protocol is planned.

### Next steps

1. Treat ELI-002 as informational for now; neither the trial nor the EAP fits unresectable metastatic disease
2. If the team still wants to ask, email [clinicaltrialinquiries@elicio.com](mailto:clinicaltrialinquiries@elicio.com) to check whether any metastatic-setting ELI-002 protocol is open or planned
3. Revisit only if the disease ever becomes resectable / MRD-state, which would change eligibility

### Trial pathways

| NCT                         | Phase           | Status                | Eligible | Central contact                                                                          |
|-----------------------------|-----------------|-----------------------|----------|------------------------------------------------------------------------------------------|
| <a href="#">NCT05716264</a> | 1               | active not recruiting | no       | —                                                                                        |
| <a href="#">NCT07083478</a> | Expanded access | unknown               | no       | <a href="mailto:clinicaltrialinquiries@elicio.com">clinicaltrialinquiries@elicio.com</a> |

### Manufacturer / sponsor contact

- **Company:** Elicio Therapeutics
- **Compassionate / expanded access:** <https://elicio.com/expanded-access-policy/>
- **Compassionate use email:** [clinicaltrialinquiries@elicio.com](mailto:clinicaltrialinquiries@elicio.com)
- **Notes:** Expanded-access requests must come from the treating physician via [clinicaltrialinquiries@elicio.com](mailto:clinicaltrialinquiries@elicio.com). The EAP is written for the adjuvant indication; a metastatic request would be off-protocol and would need the company's separate agreement.

**Payer / coverage:** Investigational; no commercial product or payer pathway.

**Notes:** The load-bearing constraint is the adjuvant/MRD setting, not HLA. Carried so the report documents why the most-discussed KRAS vaccine isn't reachable for a metastatic patient.

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## 3. INCB161734, oral KRAS G12D-selective inhibitor

*Aliases:* INCB161734

**Modality:** small\_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 monotherapy ORR 37%, DCR 78% in pretreated G12D PDAC (ASCO GI 2026) · **Geographic scope:** US sites (US call center 1.855.463.3463); ex-US sites via +800 00027423 / [eumedinfo@incyte.com](mailto:eumedinfo@incyte.com) · **Verified:** 2026-06-06

INCB161734 carries the strongest single-agent response data surfaced in G12D PDAC (37% ORR), and its

front-line trial DAWN-303 (NCT07522073) takes previously untreated metastatic KRAS G12D PDAC, so it's a directly matched first-line option from a second sponsor. The parent phase 1 (NCT06179160) also enrolls at first line and offers combination cohorts. No approval exists; enrollment is the route, with a case-by-case expanded-access ask as a fallback.

### Next steps

1. Call the Incyte US call center (1.855.463.3463) or email [medinfo@incyte.com](mailto:medinfo@incyte.com) to find the nearest DAWN-303 (NCT07522073) site with an open first-line slot
2. Confirm the documented KRAS G12D result, which this trial requires
3. Ask about the parent phase 1 (NCT06179160) combination cohorts if a DAWN-303 slot isn't available
4. If he later becomes trial-ineligible, have the treating physician email [globalmedinfo@incyte.com](mailto:globalmedinfo@incyte.com) to ask whether expanded access is possible

### Trial pathways

| NCT                         | Phase | Status     | Eligible | Central contact                                                                                                  |
|-----------------------------|-------|------------|----------|------------------------------------------------------------------------------------------------------------------|
| <a href="#">NCT07522073</a> |       | recruiting | likely   | Incyte Corporation Call Center (US); <a href="mailto:medinfo@incyte.com">medinfo@incyte.com</a> ; 1.855.463.3463 |
| <a href="#">NCT06179160</a> |       | recruiting | likely   | Incyte Corporation Call Center (US); <a href="mailto:medinfo@incyte.com">medinfo@incyte.com</a> ; 1.855.463.3463 |

### Manufacturer / sponsor contact

- **Company:** Incyte Corporation
- **Med info phone:** 1.855.463.3463
- **Med info email:** [medinfo@incyte.com](mailto:medinfo@incyte.com)
- **Product info:** <http://www.incytemi.com/>
- **Compassionate use email:** [globalmedinfo@incyte.com](mailto:globalmedinfo@incyte.com)
- **Notes:** Incyte will consider expanded access on a case-by-case basis through its Global Medical Affairs group; requests route via [globalmedinfo@incyte.com](mailto:globalmedinfo@incyte.com) and must come from the treating physician. No INCB161734-specific posted EAP protocol was found as of the verification date.

**Payer / coverage:** Investigational; no commercial product or payer pathway. Trial drug is sponsor-supplied.

**Notes:** Second-sponsor G12D-selective agent. Adds front-line breadth and a checkpoint-combination cohort relevant to the MSS immunotherapy question.

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## 4. anti-KRAS G12D HLA-restricted TCR-engineered T cells (NT-112, AZD0240, and the published C\*08:02 product)

*Aliases:* NT-112, AZD0240, HLA-C\*08:02 anti-KRAS-G12D TCR-T

**Modality:** CAR-T · **Regulatory:** investigational; no approval anywhere · **Guidelines:** none · **Geographic scope:** US/EU trial sites, but not accessible to this patient on HLA grounds · **Verified:** 2026-06-06

This is the concrete HLA-screening result the case flagged: the published anti-KRAS-G12D TCR-T works through

HLA-C08:02, and the open AstraZeneca trial (NCT06218914) restricts to C08:02 (NT-112) or A\*11:01/11:02 (AZD0240). The patient carries none of those alleles, so neither product can present his mutant epitope and he is not eligible. The KRAS-directed cell-therapy axis is foreclosed by his HLA type rather than by drug availability.

### Next steps

1. Do not pursue NCT06218914; the patient's HLA type (C05:01/C06:02, A01:01/A02:01) does not match either arm
2. If any future KRAS G12D TCR-T product restricted to A02:01 or A01:01 opens, re-screen against his actual alleles before considering it
3. Keep the small-molecule RAS(ON) inhibitors as the active KRAS axis instead

### Trial pathways

| NCT         | Phase | Status     | Eligible | Central contact                                                                                         |
|-------------|-------|------------|----------|---------------------------------------------------------------------------------------------------------|
| NCT06218914 |       | recruiting | no       | AstraZeneca Clinical Study Information Center;<br>information.center@astrazeneca.com;<br>1-877-240-9479 |

### Manufacturer / sponsor contact

- **Company:** AstraZeneca (NCT06218914; NT-112 originated with Neogene/AstraZeneca, AZD0240 is AstraZeneca)
- **Med info phone:** 1-800-236-9933
- **Med info email:** [information.center@astrazeneca.com](mailto:information.center@astrazeneca.com)
- **Notes:** AstraZeneca US medical information: 1-800-236-9933. Trial information center: 1-877-240-9479 / [information.center@astrazeneca.com](mailto:information.center@astrazeneca.com).

**Payer / coverage:** Investigational; no payer pathway. Moot here given the HLA mismatch.

**Notes:** not\_yet\_accessible is listed first because the barrier is the patient's HLA genotype, not lack of an open trial; clinical\_trial\_only reflects that the only route for any patient is enrollment. This documents case judgment call (b).

## 5. ceralasertib (AZD6738), oral ATR inhibitor

**Aliases:** ceralasertib, AZD6738

**Modality:** small\_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 2 activity in ARID1A-deficient tumors concentrated in gynecologic histologies, weaker non-gynecologic signal · **Geographic scope:** US (UCSF-sponsored) · **Verified:** 2026-06-06

Ceralasertib is the cleanest live option on the ARID1A synthetic-lethal axis, and the UCSF basket (NCT03682289) explicitly takes pancreatic cancer and assigns by ARID1A/BAF250a status. Two things stand between the patient and a slot: the trial is active-not-recruiting, so a new opening isn't guaranteed, and entry depends on BAF250a (ARID1A) protein loss by IHC rather than the frameshift call alone. The validation step is ordering ARID1A IHC before pursuing this.

## Next steps

1. Have the treating team order ARID1A (BAF250a) IHC on tumor tissue; the frameshift call alone does not confirm eligibility
2. Contact the NCT03682289 site (UCSF; PI Rahul Aggarwal) through the cancer center to ask whether the ARID1A pancreatic arm has any remaining slot despite the active-not-recruiting status
3. For drug-level questions, call AstraZeneca medical information at 1-800-236-9933
4. Keep this secondary to the KRAS axis; pursue in parallel only if ARID1A IHC confirms loss

## Trial pathways

| NCT         | Phase | Status                | Eligible    | Central contact |
|-------------|-------|-----------------------|-------------|-----------------|
| NCT03682289 |       | active not recruiting | unconfirmed | —               |

## Manufacturer / sponsor contact

- **Company:** University of California San Francisco (sponsor) / AstraZeneca (drug supply)
- **Med info phone:** 1-800-236-9933
- **Med info email:** [information.center@astrazeneca.com](mailto:information.center@astrazeneca.com)
- **Notes:** Academic-sponsored trial; AstraZeneca supplies ceralasertib. The AstraZeneca US medical-information line (1-800-236-9933) handles drug-level inquiries. No posted ceralasertib compassionate-use program for this setting was found.

**Payer / coverage:** Investigational; no payer pathway.

**Notes:** The BAF250a-IHC gate is the load-bearing constraint, alongside the trial's recruitment status. This is the more active of the two ARID1A routes (ATR vs EZH2).

## 6. daraxonrasib (RMC-6236), pan-RAS(ON) inhibitor

*Aliases:* daraxonrasib, RMC-6236

**Modality:** small\_molecule · **Regulatory:** investigational; FDA breakthrough therapy designation in PDAC; NDA expected on the strength of RASolute 302 but no approval yet (as of June 2026) · **Guidelines:** not in NCCN as an approved therapy; pivotal RASolute 302 read out at ASCO 2026 (OS 13.2 vs 6.7 mo, HR 0.40), filing pending · **Geographic scope:** US sites; Revolution Medicines runs trials in the US and internationally · **Verified:** 2026-06-06

The realistic path right now is a front-line trial: RASolute 303 (NCT07491445) takes treatment-naive metastatic PDAC and accepts the KRAS G12D under its RAS-mutant arm, with RMC-GI-102 (NCT06445062) as a backup that adds combination cohorts. There's also an FDA-authorized expanded access program, but it only opens after a prior line of metastatic therapy, so it isn't a door for him until he's progressed on first-line chemo. The one thing worth nailing down before screening is a tissue NGS G12D result to back up the ctDNA call.

## Next steps

1. Call the Revolution Medicines study line (1-844-2-REVMED) or email [medinfo@revmed.com](mailto:medinfo@revmed.com) to find the



nearest RASolute 303 (NCT07491445) site with an open first-line slot

2. Ask the treating team to confirm a tissue NGS KRAS G12D result, since the current call is ctDNA-only and tissue confirmation strengthens trial eligibility
3. If no RASolute 303 site is reachable, ask the same line about RMC-GI-102 (NCT06445062) G12D subprotocols
4. Keep the daraxonrasib EAP (NCT07573215) on file as the off-trial route for later, once he has had a line of metastatic chemotherapy

### Trial pathways

| NCT         | Phase           | Status                | Eligible | Central contact                                                               |
|-------------|-----------------|-----------------------|----------|-------------------------------------------------------------------------------|
| NCT07491445 | 1               | recruiting            | likely   | Revolution Medicines Study Director;<br>medinfo@revmed.com;<br>1-844-2-REVMED |
| NCT06445062 | 2               | recruiting            | likely   | Revolution Medicines;<br>medinfo@revmed.com;<br>1-844-2-REVMED                |
| NCT06625320 | 3               | active not recruiting | no       | Revolution Medicines Study Director;<br>medinfo@revmed.com;<br>1-844-2-REVMED |
| NCT07573215 | Expanded access | unknown               | no       | Revolution Medicines Study Director;<br>medinfo@revmed.com;<br>1-844-2-REVMED |

### Manufacturer / sponsor contact

- **Company:** Revolution Medicines, Inc.
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** [medinfo@revmed.com](mailto:medinfo@revmed.com)
- **Compassionate use email:** [medinfo@revmed.com](mailto:medinfo@revmed.com)
- **Notes:** The EAP and trial inquiries route through the same medical-information line. Requests must come from the treating physician; Revolution Medicines cannot take them from patients or families directly.

**Payer / coverage:** No commercial product yet, so there is no payer coverage to arrange. Trial drug and the EAP drug are sponsor-supplied at no charge. If daraxonrasib is approved later, coverage will follow the label.

**Notes:** Daraxonrasib is the strongest efficacy anchor on the KRAS axis for this case but is not yet approved anywhere, so access is trial or EAP only. The EAP line-of-therapy gate is the load-bearing constraint for a treatment-naïve patient.

## 7. tuvusertib (M1774), oral ATR inhibitor

*Aliases:* tuvusertib, M1774

**Modality:** small\_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO · **Geographic scope:** US / EU sites (EMD Serono / Merck KGaA) · **Verified:** 2026-06-06

Tuvusertib targets the same ATR axis as ceralasertib, but its ARID1A loss-of-function cohort (Part A3 of DDRiver 320) is built around endometrial cancer, and the relevant parts don't name pancreatic cancer. The trial is also active-not-recruiting. So this is mechanism support for the ATR route, not a slot a PDAC patient can take right now. Ceralasertib's NCT03682289 is the ATR trial that actually lists pancreatic eligibility.

### Next steps

1. Treat tuvusertib as secondary to ceralasertib on the ATR axis; its ARID1A cohort is endometrial-centered and the trial isn't recruiting
2. If wider solid-tumor eligibility is unclear, have the team check the current protocol or email EMD Serono medical information via [medical.emdserono.com](mailto:medical.emdserono.com)
3. Prioritize ARID1A (BAF250a) IHC first, since any ATR-axis entry depends on confirmed protein loss

### Trial pathways

| NCT                         | Phase | Status                | Eligible | Central contact |
|-----------------------------|-------|-----------------------|----------|-----------------|
| <a href="#">NCT05391683</a> | 1B    | active not recruiting | no       | —               |

### Manufacturer / sponsor contact

- **Company:** EMD Serono Research & Development Institute (Merck KGaA, Darmstadt)
- **Product info:** <https://www.emdserono.com/us-en/expertise/healthcare-professionals.html>
- **Notes:** US medical-information resources at [medical.emdserono.com](https://www.emdserono.com). US patient-safety / adverse-event line: 1-800-283-8088 ext 5563. No posted tuvusertib compassionate-use protocol for ARID1A PDAC was found.

**Payer / coverage:** Investigational; no payer pathway.

**Notes:** Carried as ATR-axis mechanism support. Tumor-type breadth is the load-bearing constraint.

## 8. zoldonrasib (RMC-9805), KRAS G12D(ON)-selective inhibitor

*Aliases:* zoldonrasib, RMC-9805

**Modality:** small\_molecule · **Regulatory:** investigational; no approval anywhere · **Guidelines:** not in NCCN/ESMO; phase 1 ORR ~30% in pretreated G12D PDAC (ASCO GI 2025) · **Geographic scope:** US sites; sponsor also runs trials internationally · **Verified:** 2026-06-06

Zoldonrasib is the G12D-selective agent that maps to this patient's exact allele, and the front-line trial RASolute 305 (NCT07621718) takes treatment-naive metastatic KRAS G12D PDAC, so it's the most precisely matched enrollable option on the KRAS axis. RMC-GI-102 (NCT06445062) is the platform-study backup. No approval and no standalone expanded-access program exist, so enrollment is the only route.

### Next steps

1. Call 1-844-2-REVMED or email [medinfo@revmed.com](mailto:medinfo@revmed.com) to locate the nearest RASolute 305 (NCT07621718) site with an open first-line slot
2. Confirm the tissue/ctDNA KRAS G12D result is documented, since this trial requires G12D specifically

3. If RASolute 305 isn't reachable, ask the same line about zoldonrasib cohorts in RMC-GI-102 (NCT06445062)
4. Note that the first-in-human NCT06040541 basket requires prior therapy and won't take a treatment-naive patient yet

### Trial pathways

| NCT         | Phase | Status     | Eligible | Central contact                                                               |
|-------------|-------|------------|----------|-------------------------------------------------------------------------------|
| NCT07621718 |       | recruiting | likely   | Revolution Medicines Study Director;<br>medinfo@revmed.com;<br>1-844-2-REVMED |
| NCT06445062 |       | recruiting | likely   | Revolution Medicines;<br>medinfo@revmed.com;<br>1-844-2-REVMED                |
| NCT06040541 |       | recruiting | no       | Revolution Medicines, Inc.;<br>medinfo@RevMed.com;<br>1-844-2-REVMED          |

### Manufacturer / sponsor contact

- **Company:** Revolution Medicines, Inc.
- **Med info phone:** 1-844-2-REVMED
- **Med info email:** [medinfo@revmed.com](mailto:medinfo@revmed.com)
- **Notes:** Same sponsor and medical-information line as daraxonrasib. No standalone zoldonrasib EAP was found as of the verification date.

**Payer / coverage:** Investigational; no commercial product or payer pathway. Trial drug is sponsor-supplied.

**Notes:** Distinct from pan-RAS daraxonrasib: zoldonrasib is G12D-selective and pairs to the patient's allele directly. The line-of-therapy gate on the FIH basket is the only place he'd currently be excluded.

## Unavailable (3)

### 9. MRTX1133, KRAS G12D-selective noncovalent inhibitor

*Aliases:* MRTX1133

**Modality:** small\_molecule · **Regulatory:** investigational; program discontinued · **Guidelines:** none · **Geographic scope:** not applicable (discontinued) · **Verified:** 2026-06-06

This one is a dead end. MRTX1133 was the first clinical KRAS G12D-selective small molecule, but the program was terminated for formulation problems before any efficacy read-out, so there is no trial slot, no expanded access, and no approval. The mechanism it validated lives on in zoldonrasib and INCB161734, which are the agents to pursue instead.

### Next steps

1. Do not pursue MRTX1133 directly; the program is closed
2. Route the same KRAS G12D-selective rationale to zoldonrasib (RASolute 305, NCT07621718) and INCB161734 (DAWN-303, NCT07522073), which carried the mechanism forward

**Trial pathways**

| NCT          | Phase | Status     | Eligible | Central contact |
|--------------|-------|------------|----------|-----------------|
| NCT057317206 |       | terminated | no       | —               |

**Manufacturer / sponsor contact**

- **Company:** Mirati Therapeutics (acquired by Bristol Myers Squibb)
- **Med info phone:** 1-800-321-1335
- **Med info email:** [medical.communications@bms.com](mailto:medical.communications@bms.com)
- **Notes:** Mirati is now part of Bristol Myers Squibb; the BMS medical-information line is listed for completeness, though there is nothing to dispense. Adverse-event line: 1-800-721-5072 / [worldwide.safety@bms.com](mailto:worldwide.safety@bms.com).

**Payer / coverage:** No product; nothing to cover.

**Notes:** Included so the report shows explicitly that this first-generation G12D drug cannot be obtained, rather than leaving the row silently absent.

**10. camonsertib (RP-3500), oral ATR inhibitor**

*Aliases:* camonsertib, RP-3500

**Modality:** small\_molecule · **Regulatory:** investigational; no approval; ATTACC combination trial terminated (no phase 2 expansion). Roche ended its license in 2024; Repare regained rights · **Guidelines:** none · **Geographic scope:** not applicable (relevant trial terminated) · **Verified:** 2026-06-06

Camonsertib gave ATR-class precedent, but the trial that surfaced here (ATTACC, NCT04972110) is terminated with no phase 2 expansion, didn't confirm ARID1A in its biomarker list, and didn't establish pancreatic eligibility. There is no open camonsertib slot for this patient and no posted expanded-access route. On the ATR axis, ceralasertib's pancreatic-eligible basket is the option to pursue instead.

**Next steps**

1. Do not pursue camonsertib via ATTACC; it is terminated with no successor combination trial
2. Use ceralasertib (NCT03682289) as the live ATR-axis route, gated on ARID1A (BAF250a) IHC
3. Only contact Repare directly if a clinician specifically wants to ask about any newer camonsertib protocol, since none was found as of the verification date

**Trial pathways**

| NCT         | Phase | Status     | Eligible | Central contact |
|-------------|-------|------------|----------|-----------------|
| NCT04972110 |       | terminated | no       | —               |

**Manufacturer / sponsor contact**

- **Company:** Repare Therapeutics
- **Product info:**<https://www.reparerx.com/camonsertib-atr-inhibitor/>
- **Notes:** Repare regained global rights after Roche exited the collaboration in 2024. The ATTACC combination study is terminated; no posted compassionate-use route for ARID1A PDAC was found. Any access ask would have to go directly to Repare.

**Payer / coverage:** Investigational; no payer pathway.

**Notes:** unavailable because the surfaced trial is terminated and no ARID1A-selected or PDAC-eligible camonsertib program is currently open. Included so the report shows the ATR axis narrows to ceralasertib.

**11. tazemetostat (EPZ-6438), EZH2 inhibitor**

*Aliases:* tazemetostat, EPZ-6438, Tazverik

**Modality:** small\_molecule · **Regulatory:** previously FDA-approved (epithelioid sarcoma; relapsed/refractory follicular lymphoma); voluntarily withdrawn from all markets by Ipsen on 9 March 2026 over secondary-malignancy safety signals; all trials and expanded-access programs closed · **Guidelines:** prior NCCN listings in its approved indications are moot post-withdrawal; never guideline-supported for PDAC · **Geographic scope:** not applicable (withdrawn worldwide) · **Verified:** 2026-06-06

Tazemetostat is off the table. Although it was FDA-approved in epithelioid sarcoma and follicular lymphoma, Ipsen voluntarily withdrew it from all markets in March 2026 over secondary-malignancy safety signals and shut down every trial and expanded-access program, so there is no supply route, no off-label path, and no ARID1A basket left. The EZH2 route into this patient's ARID1A axis has lost its lead agent; the ATR route (ceralasertib) is the one still standing.

**Next steps**

1. Do not pursue tazemetostat; it has been withdrawn from all markets and all programs are closed
2. Direct the ARID1A synthetic-lethal rationale to the ATR axis (ceralasertib, NCT03682289) instead
3. Note for the board that the EZH2-in-ARID1A hypothesis lost both its lead agent and its supporting efficacy signal (negative NRG-GY014)

**Trial pathways**

| NCT                         | Phase | Status     | Eligible | Central contact |
|-----------------------------|-------|------------|----------|-----------------|
| <a href="#">NCT05023655</a> |       | terminated | no       | —               |

**Manufacturer / sponsor contact**

- **Company:** Ipsen Biopharmaceuticals (US marketer of Tazverik)
- **Notes:** Ipsen withdrew tazemetostat in all indications from all markets on 9 March 2026 and discontinued all active trials and expanded-access programs. There is no current supply route. The withdrawal was driven by secondary hematologic malignancy signals in the SYMPHONY-1 trial, not by an ARID1A-specific finding.

**Payer / coverage:** No product on the market; nothing to cover. Off-label use is not possible because the drug has been withdrawn from supply entirely.

**Notes:** Status is unavailable rather than off\_label\_use: even though the drug carried FDA approval, the March 2026 worldwide withdrawal removes any supply, so off-label prescription is not actually possible. Included so the report shows explicitly that the EZH2 route is closed.

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